جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : محاسن فرحان حسين حسن
المحافظة : البحيره **الحي / المدينة :** غرب النوباريه **العنوان :** =
رقم البطاقة : 26201162502208 **رقم التليفون :** 01020778286 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 16-01-1962 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: محاسن فرحان حسين حسن
National ID: 26201162502208

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

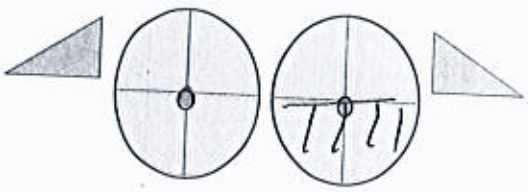
مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

٢٠٢٠

MDT FORM

Post surgery.
01020778286

Name	محاسن فرحان حسين
Age	59 years.
MRN	5803/21
ID	
Co morbidity	



Free Margin
ypT2N0

(22/6/2021) **Clinical Data**

Bx: Lt 4 Q M, received chemo (CBR) freenible	SM: Free LN / T2,N0
IHC: ER: PR: Her٢: Ki٦٧:	Mets Workup:
pT T N M	

COMMENT:

pt received 4 Herceptin before surgery

MDT OUTCOME & PLAN

17-7-2021 → for - TDM1 (completely)
- PORT
- A.I
- echo ??

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة

Pathology Report

Name: محاسن فرحان حسين حسن **Age: 59 years.**

Clinical data: Lt breast cancer, received chemotherapy

Specimen type: Lt MRM

Date (Received):22 /6/2021. **Date(reported): 29/6/2021**

Lab. Number: 1307 / 21 **Hospital Number: 5803/21**

Gross Examination:

- Procedure: left MRM
- Localization: left
- Tumor extent
  - Number: one
  - Size: 2.5x2cm
  - Skin involvement: no
- Distance from margins: least deep margin 2cm
- Lymph nodes number and size:10 nodules up to 1x1 cm

Microscopic examination:

- Histological type: remnant of IDC
- Grade II
- Lymph node involvement: FREE
- Resection Margins: FREE

Diagnosis:

Lt MRM, received chemotherapy:

- Invasive duct carcinoma GII.
- Pathological partial response (PPR)
- Free nipple
- Free lymph nodes.
- Stage ypT2 ypN0

Prof. Gamal Abdel ATY
Dr. Rana Fathy

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Med. Record No.	2020138994
Patient Name	MAHASEN FARHAN HUSEN HASSAN
Sex	Female DOB: 1/16/1962
Nationality	EGYPTIAN
Unit	External Clinic

IMMUNOPATHOLOGY - Final Anatomical Diagnosis

Hormone receptors

Specimen Id 2011250023
Accession No IMM20-5074
Collection Date 11/25/2020 8:51:47AM
Receive Date 11/25/2020 8:51:52AM

Sample Type Paraffin Block
Requesting Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

5473/20

Clinical Data

Lt. breast cancer

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark XT autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive. Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%–100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8. Comment: - There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy. - PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

left

Diagnosis :

Lt. breast cancer :
ER: Positive, strong nuclear reaction in 25% of tumor cells [6/8].
PR: Positive, weak nuclear reaction in 35% of tumor cells [5/8].

Signatura

GHADA MOHAMED MD PHD.

Pathology Report
[DOCTYPE:PATHOLOGY_REPORT]

PATIENT NAME: **MAHASEN FARHAN HUSEN HASSAN** GENDER: **Female** AGE: **58 y**	PATIENT ID #: **2020138994** ORDER #: **868083,868084** DATE OF SERVICE: **21-Nov-2020 01:21 PM**

Report: - all studies

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- Left breast diffuse edema pattern with UOQ irregular spiculated mass of high density with skin retraction.
- Associated left axillary enlarged LNs.
- This was confirmed by Complementary Ultrasound which showed left breast irregular spiculated hypoechoic mass seen retro-areolar and UOQ more around 12 o'clock with direct skin invasion, with consequent retraction, extending to the nipple, associated skin thickening reaching 8.5mm.
- No right breast solid or cystic mass lesions, apart from right 10 o'clock IMLN.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific right axillary lymph nodes.
- Pathological left axillary, level 1, 2, 3 and Rotter's, all are seen with muffled hila and diffuse cortical thickness.

OPINION:

- Left breast locally advanced malignant looking mass (BIRADS5), for histopathological correlation.
- Benign findings of the right breast (BIRADS 2)

محسن حسان

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Mahasen Farhan Hussein Hassan	Date : 06/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 63 Year(s)
Medical ID: 5803/2020	Pat. ID: 29550

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

06/02/2025 12:00 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Date : 05/03/2023	Patient Name: Mahasen Farhan Hussein Hassan
Age : 61 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
Pat. ID: 29550	Medical ID:

Name: mahasen farhan
Date: 5 March 2023

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *average in size (about 14 cm at the right MCL), smooth outline, normal echopattern.*
  No intra hepatic biliary radicle dilatation.
- *Portal vein is not dilated and patent .*

Gall Bladder: -

- *Average size , , normal wall thickness, no stones..*
- *CBD is not dilated.*

Spleen: -

- *Average size (About 9 cm), displaying homogenous echopattern.*

Kidneys: -

Are of normal size , site , well differentiated & with no gross stones or backpressure changes , no solid or cystic lesion.
-Para-aortic and pancreatic area: no gross lesion
-No ascites .

***URINARY BLADDER**: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.*

***Uterus** : is average size , normal endometrial line , no focal lesion. No adenxal mass or cyst.*

Opinion:

Normal study.

Signature Dr. Doaa Abdelaziz
05/03/2023 1:05 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

PATIENT NAME: **MAHASEN FARHAN HUSEN HASSAN** GENDER: **Female** AGE: **58 y**	PATIENT ID #: **2020138994** ORDER #: **884944,884945** DATE OF SERVICE: **05-Dec-2020 04:14 PM**

Report: - all studies

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER

1-CHEST

- The left breast shows a large enhancing lesion with spiculated margins measuring about 4.1 x 4.1 x 7.7 cm in its maximum AP x TS x CC dimensions respectively, and seen infiltrating the overlying skin reaching the nipple, with associated edematous changes and skin thickening.
- Multiple enlarged left axillary lymph nodes in level I and II, with the largest seen measuring about 18 x 15 mm in its maximum axial dimensions.
- Multiple bilateral pulmonary nodules are seen scattered in both lung fields, the largest is seen in the middle lung lobe measuring about 11 mm.
- Multiple right hilar, pretracheal, prevascular, paratracheal, retrocaval and subcarinal lymph nodes, the largest is seen measuring about 11 x 12 mm in its maximum axial diameter
- No gross cardiac abnormality.
- No pleural or pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous hypodense CT texture denoting fatty infiltration. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen. A small splenule is seen medially measuring about 13 mm in diameter.
- Apart from left extra-renal pelvis, Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenal

CT Scan
[[DOCTYPE.OT_SCAN]]

glands.

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.
- The examined axillary and appendicular skeleton shows moderate forward slippage of L4 over L5 vertebral bodies associated with bilateral pars breaks.

Opinion:

- **Left breast locally advanced neoplastic process with left axillary lymphadenopathy.**
- **Multiple bilateral pulmonary nodules and intra-thoracic lymphadenopathy, likely metastatic.**

Reported By,

Dr. Amr Farouk ,MD
Dr. Ahmed Abdelmoneim, MSc **06-Dec-2020 12:13 PM**